

COMPANY DEEP-DIVE

Former Senior Director at Roche Skeptical About QuidelOrtho's Competitive Positioning and Expansion Challenges

DATE PUBLISHED	INTERVIEW DATE	EXPERT PERSPECTIVE	ANALYST PERSPECTIVE	PRIMARY COMPANY
03 Feb 2026	20 Jan 2026	Competitor	Investor-Led (Buy-Side)	QDEL

SMART SUMMARY

Summary

The interview covers QuidelOrtho's competitive positioning, product development strategy, challenges in point-of-care and molecular diagnostics, international expansion, and regulatory hurdles. The discussion also addresses the impact of post-merger integration on service and support, as well as the company's vulnerabilities relative to larger industry players.

EXPERT BACKGROUND

- Former Senior Director, Development Functional Partner with extensive experience in product development and registration support for diagnostics platforms, with direct knowledge of industry competition, platform development, and regulatory challenges.

EXPERT SENTIMENT

- Skeptical about QuidelOrtho's ability to compete with larger players due to limited high-throughput offerings and a narrow focus on point-of-care molecular diagnostics.
- Believes QuidelOrtho faces significant challenges in expanding its menu and achieving commercial viability with new platforms, especially compared to established competitors.
- Views the company's post-merger integration and international expansion as necessary but slow, with ongoing hurdles in regulatory expertise and service support.

TAKEAWAYS

- QuidelOrtho has struggled to become a major player in high-volume, high-throughput diagnostics, with its merger and acquisitions aimed at expanding into new segments but facing delays and challenges.
- The company's focus on point-of-care molecular diagnostics is seen as limiting, with significant competition and commoditization in the market, and difficulties in expanding test menus beyond simple respiratory pathogens.
- QuidelOrtho's international expansion relies heavily on Ortho's distribution network, but integration and effective global sales are expected to take several years.
- Post-merger service and support may have suffered due to consolidation, potentially impacting customer satisfaction, especially as support structures are reconfigured.
- Regulatory and quality system challenges are more pronounced for QuidelOrtho compared to larger incumbents, particularly in navigating complex international requirements and building trust with regulators.

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Expert Bio

Prior to D&L Consulting, the expert was the Senior Director, Development Functional Partner at Roche, leaving June 2024. Spent more than 25 years developing molecular IVD tests and therefore am quite familiar with Quidel's products in that area (although they've been traditionally a minor player especially comparing to giants like Roche, HOLOGIC, BD and Abbott. Chief Operating Officer at D&L Consulting, Education and Services LLC (self-employed). Reporting to firm partners/clients. The expert is responsible for founding and managing a consulting firm specializing in molecular diagnostics, medical devices, and the IVD industry. Responsibilities include leading client engagements, providing operational and R&D strategy guidance, coordinating a global expert network, advising startups and incubators on design control and post-market support, and conducting regulatory submission reviews, R&D audit preparedness, and M&A due diligence. Prior to D&L Consulting, the expert was the Senior Director, Development Functional Partner at Roche, leaving June 2024. Reporting to senior R&D and lifecycle leadership, the expert was responsible for influencing lifecycle strategy, overseeing development and post-market support of more than 20 IVD assays and three diagnostic platforms, managing and developing 80–120 R&D staff, leading FDA and ISO engagements, managing CHF 3–12M budgets, and driving cross-functional initiatives to improve customer experience and integrated evidence planning. The expert can speak to and has implemented IVD product development, molecular diagnostics platforms, design control and design transfer processes, FDA and ISO regulatory engagement, post-market surveillance, integrated evidence planning, R&D talent development, and large-scale cross-functional R&D operations.

Interview Transcript

Client ▶ 00:00:00

Thanks for the call. I am interested in learning more about Quidel. To get started, let's discuss where QuidelOrtho wins and loses versus large players in the space. Where do you think QuidelOrtho currently wins and loses bids versus peers like Roche, Abbott, BD in core lab chemistry and immunoassay tenders? What specific features do you think tip the scale?

Expert ▶ 00:00:23

Unfortunately, I don't think they ever were. Even after the merger with Ortho or Quidel, ever really was a major player. **They were part of competition in the industry**, but they never really played in the high-volume, high-throughput segment at all. I think I saw their merger with Ortho was an attempt to get into that maybe through Ortho's portfolio in blood testing, blood supply, but that didn't quite happen. Even what they called Savanna, I think, was a molecular class. That was more of a point of care.

It was earlier in 2025, when they announced that they going to stop that they will acquire LEX Diagnostics. I think that's the name of the company, and go with that instead. That is also point of care. Generally, they are player. They are trying to become a player in point of care, point of contact segment for molecular, but they have ways ahead. Especially, when they spend a few years developing one platform, then they stop, and then they restart with another acquisition.

Again, from what I understand, it's not really even complete yet. It's hard to tell where are they going to go. I don't think there's such a serious competition to heavy hitters like Hologic, Abbott. They're probably trying to hire some talent from Abbott. They hired their head of diagnostics, I believe, recently as well. Was it in 2024 or something?

Client ▶ 00:01:56

Interesting. Recognizing that QuidelOrtho is a much smaller player in this space, do you think they differentiate in terms of finding their niche? I'm curious as to what competition in the space looks like more broadly. For example, does it tend to be more based on pricing, technology, service responsiveness?

Expert ▶ 00:02:13

Yeah. That's what they will have to compete on, either their prior base because they do have prior customer base from Quidel, at least in the molecular diagnostics, immunoassays, antigen testing, but not really clinical chemistry. I thought they were trying to, by merger with Ortho, venture more into that because Ortho has a platform, but it was more geared towards blood safety. Maybe that's their attempt, but they don't have any molecular diagnostics platform that I know that is a medium to high throughput. It's all lower volume point-of-care.

Granted, that's a business, but everybody seems to be getting into point-of-care business these days. It started during the pandemic or post-pandemic because obviously, there was such a need and there was an impression, a lot of people's mind that that's where everything is going to go. I don't believe so. There's going to be some business for it. **It's going to be significant, but it's also going to have its limitation.** I can try to explain why if that's of interest, push on you something.

Client ▶ 00:03:21

Yeah. If you could, that would be great.

Expert 00:03:25

There's certain testing, where the patient journey and the need for diagnostics basically almost ends with the diagnosis. If it's a typical run-of-the-mill respiratory infections for which there's either no need for any real treatment or there is maybe for specific populations there may be, but that's it. You collect the sample, you test it quick, you get the result quick, the doctor may prescribe something or say, "No, you don't really need a prescription," and that's it. That's the end of the story.

For testing, where that testing is one-time deal in the continuum of care, that's a different story. Take virology, HIV, HCV, HPV, you need a treatment, and then you're going to go in treatment, and then there's going to be more testing, and it's going to go on and on. There's no need to know result right away. That's not going to go away.

Take HPV cancer screening. That's a whole continuum in the journey for the patient. Just because you tell the patient that she has HPV, that's not the beginning, not the end. There will be more testing. They'll have to be plugged into the whole continuum of care. You cannot just do it at the point of care. It just makes no sense. That's high volume. It will not go away.

Even STIs, the sexually transmitted danger like CT and G. You can argue that some people willing to pay and get tested because something happened last night and they don't feel good or they suspect something, but again, they would still need prescription, they will still need to come into the context of the healthcare system and go on from there and then probably get tested when things are done.

Again, that one being completely point of care is also kind of question. **These are the limitations of that segment**, in my opinion, at least for typical run-of-the-mill test. Don't get me wrong, they both have their share and they will develop. There is no doubt that point of care will grow and probably maybe faster than the high throughput, but to say, some people try to say that it's going to replace centralized testing, that's just extremely difficult to do.

Client 00:05:43

Are there certain customer segments where you think Quidel does punch or could punch above its weight?

Expert 00:05:49

Again, in what they're trying to do for molecular, from what I see, they're trying to do is, again, get in a point-of-care that would be walk-up clinic, urgent care center, community-type of center, maybe even pharmacy. There are some opinions that the big retailers like Walmart and even Amazon may go into the business of being provider for diagnostic tests and drugs. Walmart already has it. They have pharmacies over them. **I think they even experimented with urgent care walk-up clinics, Walmart that is, but they weren't very successful.**

That's where the type of instrument that I see Quidel trying to develop was definitely they would have a space now that's provided that they can deliver a menu. **That is a tricky thing** because if you ask me, looking at their current LEX pathway forward, where again, from what I can remember, I thought that whole deal of the acquisition of LEX was pending the LEX Diagnostics receiving FDA approval, or the usual run-of-the-mill test SARS Flu A and B. First of all, I don't believe they have received it yet. It's been six months.

Now, not the end of the world, first-time platform, first-time submission to the FDA. I think they have a greener team. **Not yet an indication of a disaster**, but it's long. From what I understand, the deal is not a deal even until that clearance is obtained.

My next point is that's a relatively easy test to do no matter what you claim. It's easy to collect and process sample. I believe, they claim that they don't need essential sample preference. They say swap to result. Most people can do it. It's a very easy sample. Now, try to do that from a whole blood sample. Try to do it from stool, gastro pathogens. That's not an easy thing. Not on the cheaper platform that may not even have a front-end grade extraction technology, which that's what it looks like to me.

They're going to have their challenges. Just because you have again something that everybody does and SARS, that's not an indication that is a successful and manufacturable and commercially viable platform that will eventually have the menu that the customer. That's my take. Maybe I went too far above your question.

Client 00:08:17

Not at all. This was helpful. Going a bit further into this, Quidel markets dry slide clinical chemistry, so using heritage Kodak technology that only one other vendor offers. Do you think that specialization meaningfully differentiates service levels, or do you think it's becoming commoditized?

Expert 00:08:37

That is commodity, and how you do it is not really all that important. Like you said, it's a legacy technology from what I can tell. Did they buy something? I don't remember. Maybe they bought something from Kodak. I don't think that distinguishes them per se.

Client ▶ 00:08:59

Got it. In molecular point-of-care, Quidel is betting on the LEX Diagnostics acquisition for faster, cheaper, menu expansion. From your external view, how credible is that timeline compared with other near-patient molecular roadmaps?

Expert ▶ 00:09:15

It's a typical promise from a start-up to a bigger buyer. It's not very credible. That's what I meant by saying, "Hey, look, they submitted in June of 2025. They still have not gotten FDA approval. Again, you can make an argument that's because it's the first time, and FDA needs more time to understand the platform.

Again, from what I could tell, there's no real magic to it. Yes, they still do PCR. Yes, they do it very fast because it's in the cartridge, it's in smaller volume, and because they don't really do any nucleic acid extraction in front of it. That's why it's fast. That's it. **That's the limitation for them** that I think they're going to have a hard time really building a menu once they hit the more challenging.

You can look at the histories of most of the similar acquisitions. When the big diagnostics players acquire a smaller, promising technology, especially cartridge microfluidic base, it takes a lot of effort on the part of that major manufacturer to really turn it into commercially viable. By viable, I mean that making of that cartridge and then selling it at the lower price because that's where you want to compete, is giving enough margin. That's the challenge. The second one is adding the menu once you go past the typical run-of-the-mill simple respiratory pathogens.

Client ▶ 00:10:46

Got it. The QUICKVUE Influenza+SARS Test targets pharmacies and urgent care clinics. Have you observed any share shifts or pricing pressure in these channels since the September 2025 launch? Has it been any part of any conversation? I'm just curious as to what impact that might have.

Expert ▶ 00:11:06

It's hard for me to tell because again, I'm not on the commercial side of things. I'm product development registration support. Again, from a technology standpoint, those are just typical antigen test lateral. There's nothing magical about it. There's so much competition in that field. Find it hard to believe, but that changes things.

Client ▶ 00:11:29

Got it. It sounds like competition is just intensifying post-COVID normalization.

Expert ▶ 00:11:36

Yeah, because there's not much market left. People are buying it, of course. It's also seasonal. You need to launch that at the right time. There's no point of trying to roll it out and make a big deal in noise when it's springtime. They probably timed it correctly, but **it's going to be really tough to take** a whole lot there because there is a lot of competition and the price is already get down. You can go to any pharmacy and buy this. They're going to have two or three for you to choose from. People don't really understand or care about what's inside and who made it. They get the price.

Client ▶ 00:12:20

Interesting. Shifting gears slightly, I'd love to touch on the international footprint. QuidelOrtho's global reach spans 130+ countries. How does its international distributor's heavy model compare with other direct sales footprints in EMEA and emerging markets? How important is that? I know this is not tied super closely with the work you were doing, so if it's not a question you can speak to, we can move on to the next one.

Expert ▶ 00:12:47

Quidel, traditional within U.S. company. They really did not have much outside U.S. until they merged with Ortho. I think that was part of the merger strategy because Ortho is a global company. From what I could gather, that merger was partially to use Ortho's network distribution and support sales worldwide to give Quidel access to it.

Now, it's been what, I think, it was 2022, so it's only less than four years. From my experience, for a larger company to incorporate and start distributing a newer acquisitions product, it probably takes a couple of years. They're probably at best start generating real revenue internationally two years down the road. Again, with what? It depends. I wouldn't have knowledge, but if they were trying to do it with their bread and butter at the time, SARS and flu test, that's over. That's just endemic now, not pandemics. There are some demands, so it's hard for me to tell how successful they are, but this was the right idea.

Maybe, to help elaborate why I think it's not an easy thing, especially when it's a different type of programs, if Ortho's been traditionally in

clinical chemistry and blood-typing business, their distribution, their salespeople, their support people are trained on that. Now, you add some molecular products or even antigen test, that's all new. It takes time to bring people up to speed, to train them, and have them successfully go and take business from competition. That makes it even more challenging, not impossible. Again, I think, overall, Quidel has been limited to U.S. That was the right strategy. It's going to be a while.

Client ▶ 00:14:37

Public filings show **Quidel's point-of-care revenue fell to \$165 million** in Q3 of 2025, while core, ex-COVID grew slightly. Do you sense that Quidel defending share through discounting, enhanced service, or contract bundling? What do **you think is contributing to that revenue drop?**

Expert ▶ 00:14:59

Again, I think **it's just overall drop in demand post-COVID** and them not really having much else to offer. Everybody experienced how different players anticipated and planned for it. That's different for different, and the bigger ones, of course, as always have more capacity to absorb that decline because everybody knew it would decline. **It was very hard to predict** how fast, and it was not the same across the globe. Again, how the companies reacted to it, whether or not it will happen.

Client ▶ 00:15:29

Do you have insight in terms of hospital purchasing of integrated offerings versus one-off? For example, how hospital labs evaluate Quidel's integrated offering of immunoassay, clinical chemistry, and transfusion medicine versus a best-of-breed multi-vendor strategy?

Expert ▶ 00:15:47

Help me understand the multi-vendor strategy.

Client ▶ 00:15:51

I was just saying Quidel has an integrated offering across its different focuses, such as the clinical chemistry and the transfusion medicine. I'm curious as to how hospitals choose between that, and then doing a multi-vendor strategy where they're choosing the best vendor for a given diagnostic or given lab.

Expert ▶ 00:16:08

Big hospitals who can afford that, first of all, because not everybody does. The bigger hospital that are part of a big healthcare systems. It actually try to source as much from the single vendor as possible because that gives them a lot of bargaining power. Quidel's Ortho portfolio, **Quidel themselves were not really competition with QuidelOrtho**. That's a little bigger because yes, now, you have blood typing, you also have molecular clinical chemistry. That's better positioned for them, but they're still missing other pieces compared to someone like Roche basically offers pretty much anything to everything.

Those are also slightly different parts of even hospital system, blood typing. Do they even need to do it, or they go to blood banks, or they work with larger vendors? They may not even need that part. Here comes QuidelOrtho and say, "Well, here, we have this end and this end. Would you be interested?" They can be attractive. I imagine that **they will be less** attractive than again someone who has a wider portfolio, like Roche. I hate to brag about. That's reality of it, just over the years. It's such a monster everything.

Client ▶ 00:17:30

Recognizing QuidelOrtho is still smaller relative to Roche and other players. Have you heard of or seen QuidelOrtho leveraging its post-merger scale to negotiate, reagent rebates, or analyzer placements that even smaller players may struggle to match?

Expert ▶ 00:17:48

Again, that's not the part of business that I have a lot of exposure. I'm sure they do. They not very smart not to try. Is really a question of how successful they are. Now, again, I'm sure Ortho had their customer base, and they tried to now push that "Hey, now we have that part of Ortho **that may be more successful**. I guess it's now company." Yes as a company now they push it. How successful, that's not really something I can have an insight.

Client ▶ 00:18:17

Which product category, immunohematology, donor screening, or molecular, do you think **Quidel is most vulnerable** in over the next 24 months, and why?

Expert 00:18:27

Donor screening is big. They offer something in blood in donor screening, but not everything. They do a lot again from Ortho typing, some compatibility, but they don't do like molecular testing on the blood safety. Again, in that field, there's really only two players Roche. That's fairly position.

On the molecular, again, **they're still vulnerable if you ask me** because the menu is very short, and still, that's more of a point of care.

How much hospitals prefer to do it. Large hospital system, they have their own laboratories that usually run like a factory on their own. Some of the smaller hospitals also, what they do is they send their samples to the LabCorp and Quest and other big players outside U.S. They are outside U.S. rather than do their own and buy instruments and services for someone.

Client 00:19:22

Got it. With that, what would you do if you were in QuidelOrtho's position to either differentiate yourself or secure your share or establish some competitive mode?

Expert 00:19:33

They would try to further diversify what the offering is and not just try to capitalize on what Quidel could do and then what Ortho used to do. You really have to have a broad offering in all domains. That's not easy. That's a lot of money. That's a lot of investment.

Honestly, I think, they're such a narrow focus on the point-of-care molecular as if that is somehow differentiating feature of that so-called-ultra-fast PCR. Again, at the end of the day, the customers don't really care what's underneath and what kind of technology there is. There are ways to get your answer at the point of care with molecular with almost PCR, like accuracy within 10, 15 minutes, 18 minutes or something like that. There are others who offer that.

Even Roche on the Liat platform offers much broader menu and it is. "Okay, it may be not 10 minutes, it may be 20 or 18 minutes,' but if you think about it, how much difference does that make? Does that patient really going to walk away because it's 10 more minutes? Kind of hard to.

Client 00:20:44

Are there niche or other small players you know of in the space that you think have done a good job of differentiating themselves and carving out part of the market for themselves?

Expert 00:20:53

Those who did, eventually end up being acquired. Take GenMark, they were very interesting, and for a while, they had a market share, at least in the U.S., they had significant market share in the multiplex testing, but they ended up being acquired by Roche, partially because of that. I think Roche is a way to get rid of one of the competition and acquire know-how and technology.

The smaller players, I think, in that space, they're only successful up to the point at which they are either being acquired or trying to merge. I think QuidelOrtho or the merger is an attempt to get there not through being eaten by a big player like Abbott or Roche or Danaher for that matter because if you look at Danaher, the way they do it, they're not even a diagnostics company strictly speaking, but they acquire bits and pieces. It's a holding.

Client 00:21:46

Going back to R&D and platform life cycle. Where do you think QuidelOrtho underinvests relative to larger players like Roche or Abbott?

Expert 00:21:56

It's hard to tell whether they underinvest. I think they're narrowing themselves a little bit again with this switch back and forth between what was it, Savanna to LEX. You still were doing something PCR-like, and you're now doing another PCR-like, but **you lost \$150 million in the process.** Technologically, this isn't something really that new at all, by this time. Again, **they are going to have a challenge** once they go to a more interesting sample-type, which they will have to in order to expand the menu.

If you ask me, the chances are they're going to run into the same thing that they cannot deliver the menu without significantly redesigning the platform, which is, again, **that's costly, and that will slow it down.** The whole value proposition of saying, "Look, you're going to get a PCR-like result in six to 10 minutes." It's going to just evaporate right there. I think by saying under investing, it's not maybe the right way of looking at it. They're not investing in anything that's really that revolutionary.

Client ▶ 00:23:04

Got it. How important is post-market life cycle management and diagnostics really? How well does Quidel do this versus larger incumbents, just based on what you've heard from conversations or discussions in the space?

Expert ▶ 00:23:18

It is very important. A lot of customers, they definitely considered that as part of the package when they make decisions which vendor to go with because it's not really a matter of, if something will go wrong, it's when. It will require service, **things will go wrong with reagent lots every now, and then there will be some mutation** that will pop up and the vendors need to adjust and react for it, and how well they manage it is very important to the end user.

From what I heard, traditionally, in the older original Quidel, they were pretty good with support, but that's going by memory by talking to customers that I know. What they are now after the merger with Ortho, that's harder for me to tell, actually. I haven't had enough experience interaction with them.

Unfortunately, usually, those mergers make a dent on the service because, again, it's now consolidating all the support structures and cross training field service, application field service engineers, people who take complaints on first line, second line of support that usually takes a hit, then they may recover and be quite well, and with the larger company and maybe better access, that may be a better proposition, but it may go the other way as well. I don't know if Ortho, Quidel went as far as consolidating, where their support people sit, and how they handle that part, but if they did, again, **that usually may add to customer frustration** rather than make them feel better.

Imagine you're the lab, and you source your diagnostics to instruments and reagents from Quidel, and then now it became QuidelOrtho, but you used to know your local FSE, or regional FSE **who would show up every time something wrong with the instrument**, you knew him on the first name basis, in most cases.

Now, all of a sudden, they give you an 800 number, and it's someone sitting somewhere in Costa Rica or something, and they try to route you through many channels before, another guy shows up or maybe even the same guy shows up or a gal, but you just spend that much time and energy, and before, you didn't have to do it. That's a scenario where it does. It doesn't have to be that way, but I know it happens.

Client ▶ 00:25:35

Got it. From a regulatory and quality systems perspective, where do you think **smaller players like QuidelOrtho tend to struggle compared with giants like Roche?**

Expert ▶ 00:25:46

It's getting and retaining the right expertise. It's absolutely a challenge. A lot of times, the smaller players in the beginning, they rely on external help. They hire literally, there is a fair number of consultancies and individuals who help with submissions and all of that, but **they may or may not be as good. That's where the advantage of the bigger players who have a larger base of experts and more experience and more reputation** too. Reputation with regulator is a significant deal, not only during the submission, but also the post-launch.

I can tell you that there is certain trust built over decades between the larger, and **it's not in the wrong sense**, but there is some trust and relationship being built between the large players, regulatory people, and agencies. It's worth a lot. There may be higher expectations too. Sometimes, that's also true. There seems to be more scrutiny on the larger, more established players than on the smaller, at least that's how it used to be. Of course, the pandemic and EUAs and all of that would change landscape a lot, but I think, at least FDA came back significantly on the scrutiny after pandemic was over.

In Europe, **that's even more challenging in a way** because I'm sure you're aware they switched complete. They did away with IVD Directive. By now, there are no more products that are allowed to be sold that are not IVD-compliant. That compliance requires basically registration with notified body, which is not one as in U.S., so that makes it more complex those different notified bodies. You have a choice which one to use, but they also have their own specific works if you wish and what they expect.

For smaller companies **that's more difficult to learn and choose and choose wisely**, which notified body to go with. For someone like Quidel, who did not have a lot of ex-U.S. experience, that's a barrier. Is it unsurmountable? No, but it takes time, and that translates into.

Honestly, I don't even know how much experience they would have with regulators, let's say, in Australia because TGA is its own beast, more like FDA but has its own. Health Canada also has their own works. You try to go to Japan, good luck. That's pretty strange in there, MHLW. PMDA is in China, but that is a completely different piece. Quidel have order aspirations in China. I'm not aware of.

Client ▶ 00:28:31

Got it. It'll be something to look out for then. From a molecular diagnostics perspective, is **Quidel structurally disadvantaged in molecular** because it's historically been a minor player, or do you think that gap is realistically closable? For example, is the barrier technology, is it installed-base customer trust. I'm curious as to what would eventually.

Expert ▶ 00:28:54

I would say they are, and they would be even more, since, again, they're going to be rolling out new platform with some loud claims, but it would be new to people. Last I checked, I have to say, I had to refresh my memory and I checked on LEX Diagnostics, and they make this large page statement, where they have 50 people working on developing it. That's not a whole lot of people to develop a platform and even a single assay. I think they say 55. That's not a whole lot actually.

Even though it's a smaller instrument, but it's not trivial. I think **they are quite disadvantaged in that sense**. They could compete for brains, so to speak, and lure people from more established manufacturers. **They probably market is pretty tough** right now, and the bigger players are really, really cost-conscious, and they sometimes, were downsized indirectly. They're not really downsizing, but they're very strict. They're watching their budgets and their headcounts very close, not hiring.

There is talent out there that Quidel could potentially tap into. I'm sure they try to do it, but it takes time. It's also interesting that **they decided to abandon that Savanna platform**. Did they switch the brain power from that was dedicated on working that merged in with LEX Diagnostics people working? That's not trivial either.

Client ▶ 00:30:22

Appreciate the detail there. Thank you, and have a great rest of your day.